Draft for discussion

Meridian Health: closing the £38m gap and delivering the new surgical wing

Diagnostic findings, savings plan and capital programme status

Meridian Health Group Board | 18 November 2026 | Transformation Office

Confidential and proprietary. For internal discussion only.

Four numbers explain most of the deficit

£38m

Recurrent deficit, FY2025/26

Up from £21m two years ago

£812

Cost per bed day

6% above plan; peer median is **£744**

£31m

Agency staff spend

38% above plan, concentrated in three wards

71%

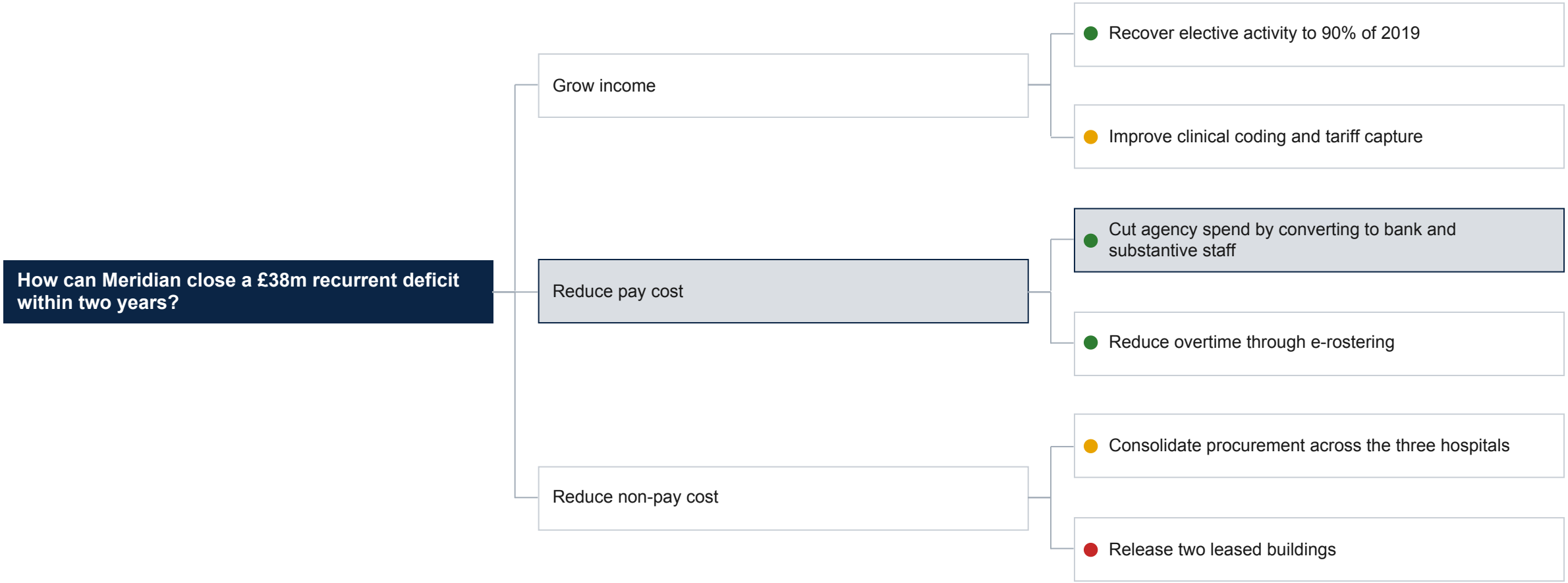
Elective activity vs 2019

Lost income of about **£14m** a year

Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

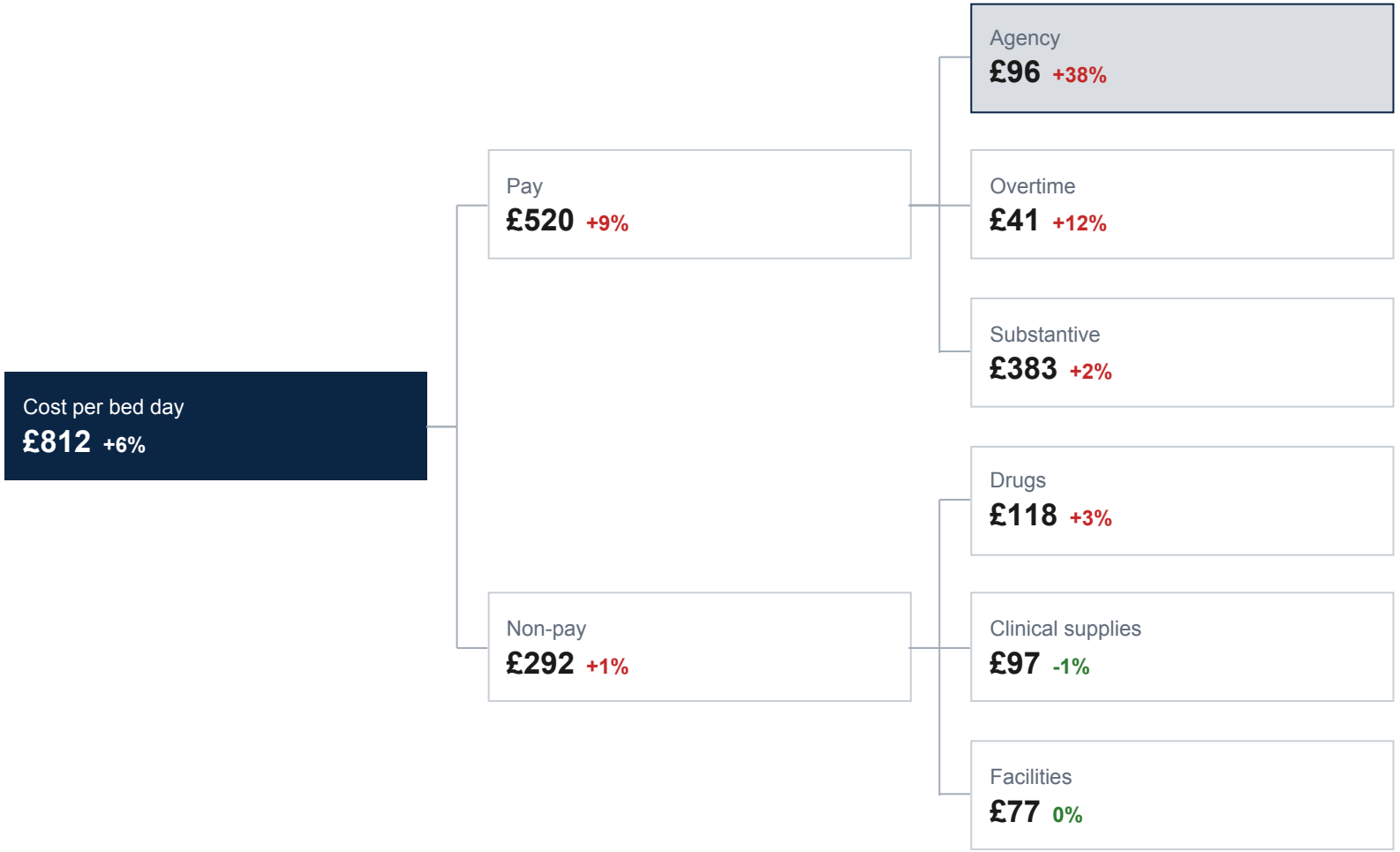
The gap can be closed through income, pay and non-pay; pay offers the largest single lever

● Supported ● Partly supported ● Not supported



1. Dot shows our assessment of each hypothesis after the diagnostic: green supported, amber partly, red not supported.
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Cost per bed day is £812, and agency staff explain most of the overspend



What the tree shows

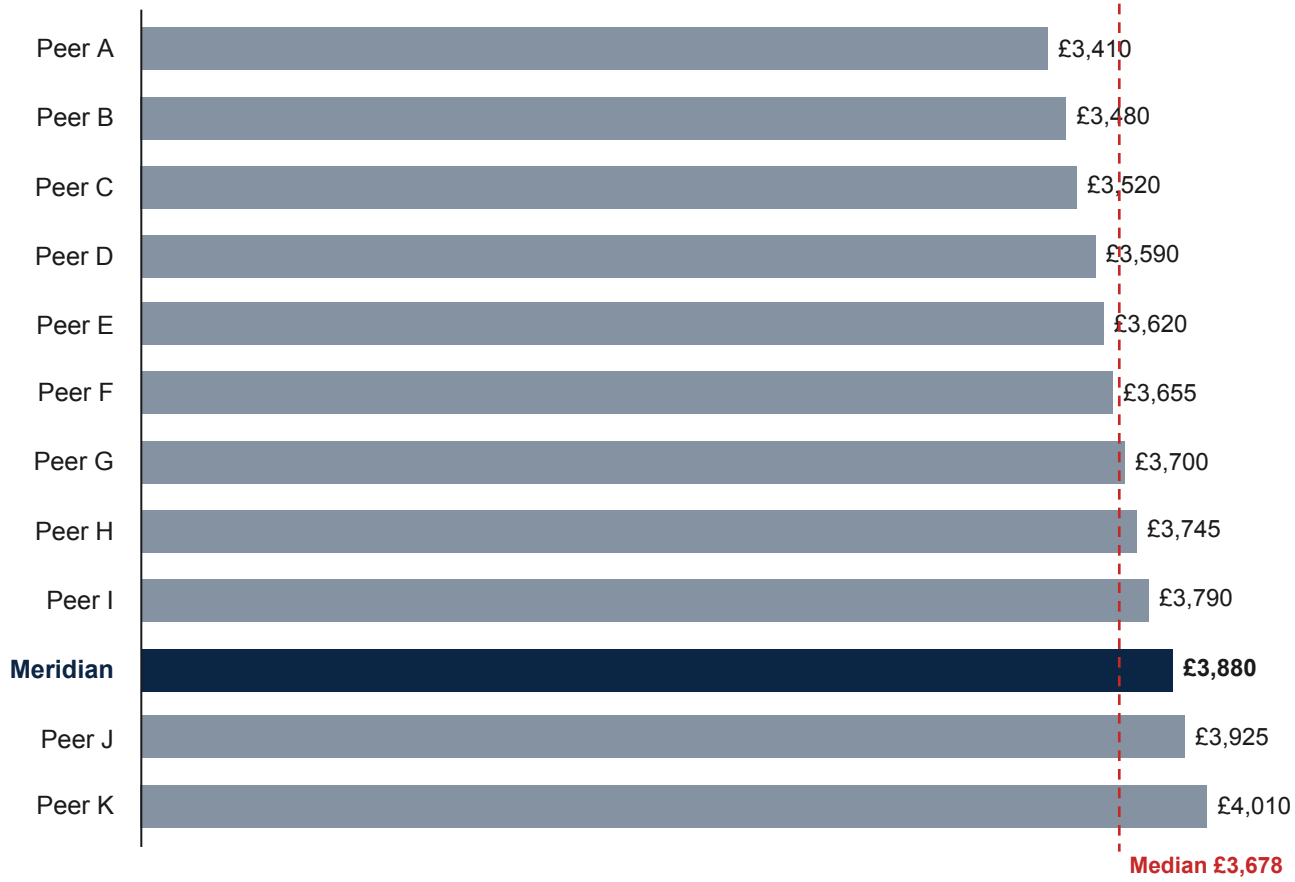
1. Agency staff add **£96** per bed day, 38% more than planned.
2. Non-pay is broadly on plan; the problem is pay.

1. Change is against the FY2025/26 plan.
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Meridian's cost per weighted activity unit is the third highest of 12 peers

Cost per weighted activity unit (WAU)

£, FY2025/26



Implication

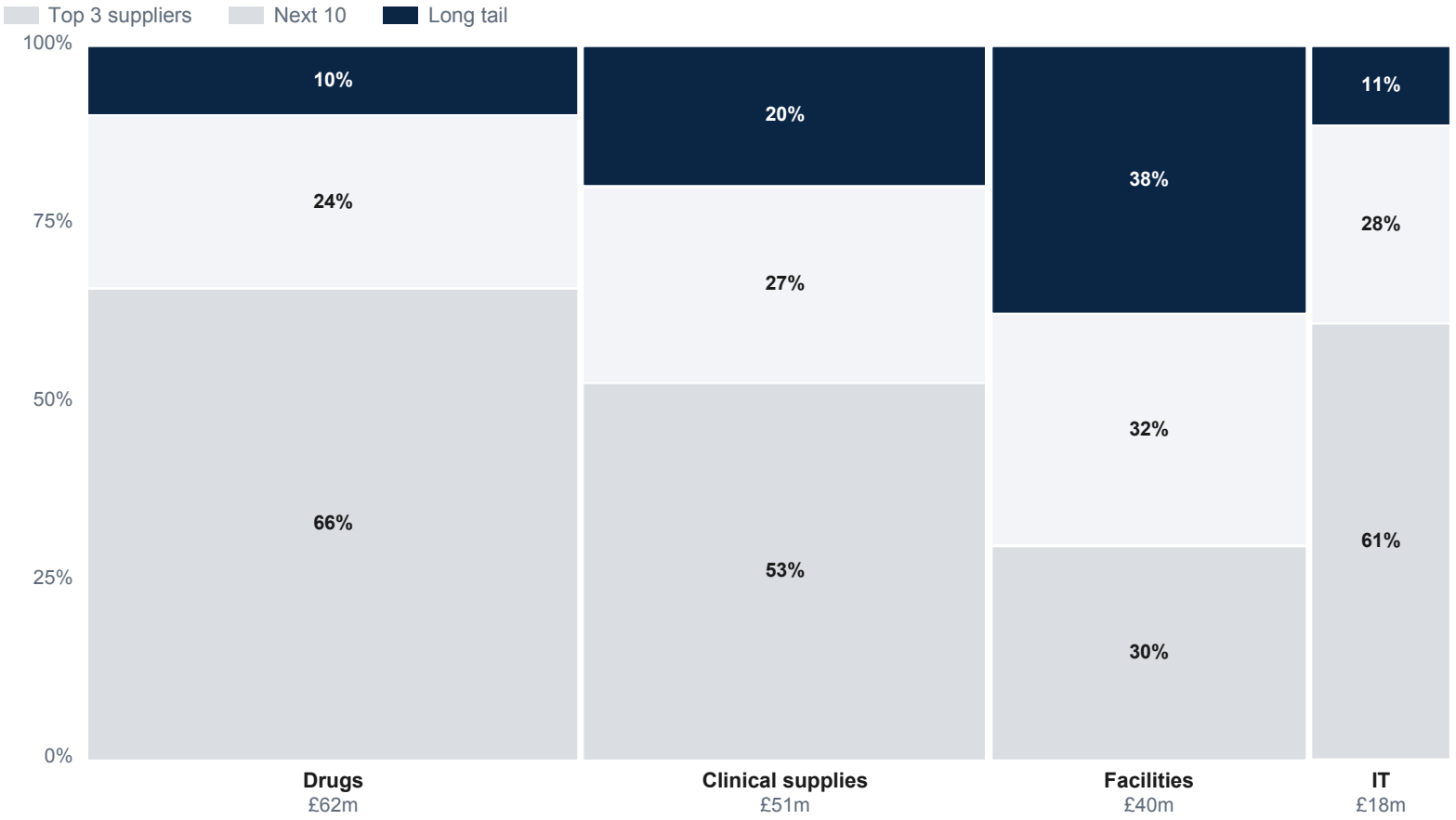
1. Reaching the peer median would save about **£9m** a year at current activity.

1. Peers are acute trusts of similar size and case mix; FY2025/26 published accounts.
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Non-pay spend is concentrated: three suppliers hold over half of clinical supplies

Non-pay spend by category and supplier concentration

Column width = annual spend; height = share of category spend



Where to act

1. Facilities has the longest tail: 38% of spend sits with small suppliers.
2. Consolidating it is the quickest non-pay saving.

Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Staff and managers describe the same three problems

Rostering

“We fill gaps with agency because the roster is finished four days before the shift, not six weeks.”

– Ward manager, surgery

Rostering

“Overtime is approved by whoever is on shift. Nobody sees the total until month end.”

– Finance business partner

Theatres

“We lose a theatre list most weeks because the patient wasn't pre-assessed.”

– Consultant anaesthetist

Procurement

“Each hospital buys its own gloves. We have eleven contracts for the same product.”

– Procurement lead

1. From 34 interviews in September and October 2026. Fictional quotes for this example.
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Five changes to the operating model address the root causes

	Today	From April 2027
Rostering	Paper rosters, finalised four days ahead	E-rostering, published **six weeks** ahead
Temporary staff	Agency booked by wards directly	Central bank office; agency only with director sign-off
Theatres	Lists planned a week ahead; 14% cancelled on the day	Pre-assessment two weeks ahead; target **5%** cancellations
Procurement	Three hospitals, 11 glove contracts	One group category team; one contract per product
Performance	Monthly finance pack, five weeks after month end	Weekly ward dashboard on pay and activity

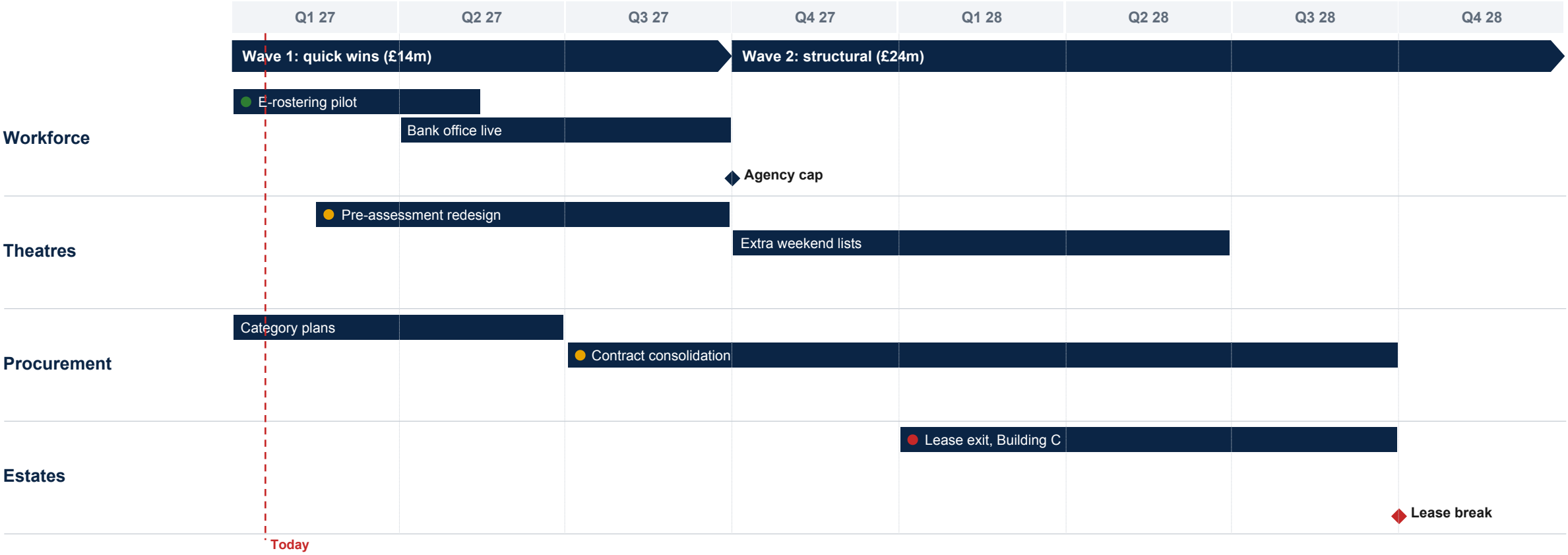
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

We will deliver the programme in four stages, piloting on three wards before scaling

Diagnose	Design	Pilot	Scale
• Baseline cost per bed day by ward • Agree savings targets with each division	• E-rostering rules and bank office • Procurement category plans	• Three highest-agency wards • Weekly tracking against the baseline – Go or no-go at week 12	• Roll out to all 42 wards • Hand over to business as usual
Owner: Finance	**Owner:** Transformation Office	**Owner:** Divisional directors	**Owner:** COO
Duration: Done	**Duration:** 8 weeks	**Duration:** 12 weeks	**Duration:** 9 months

Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

The savings plan reaches £38m by March 2029, with workforce changes delivering first



1. Indices are quarters from Q1 2027; savings are full-year recurrent.
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

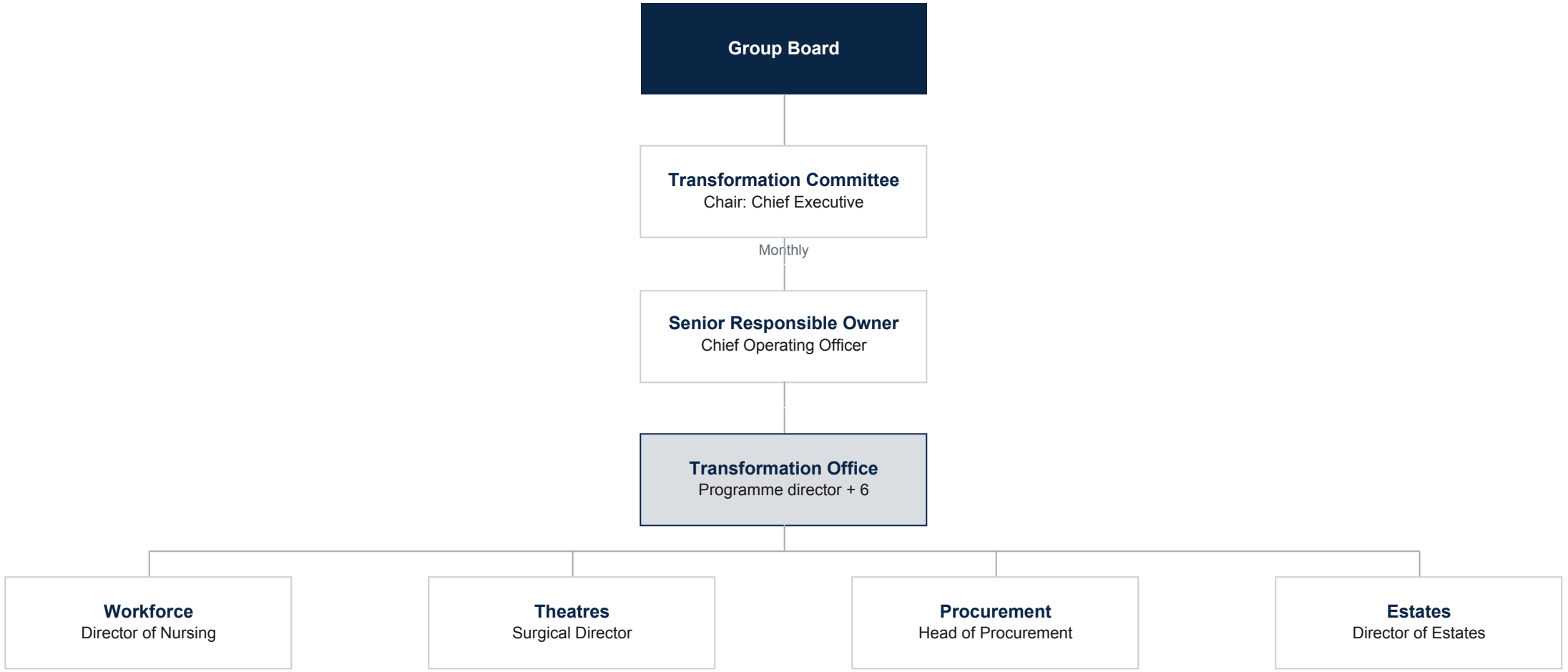
Key milestones for the next eighteen months



● Complete ▭ Planned

Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

The Transformation Committee owns delivery; one SRO is accountable to the Board



Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Every savings decision has one accountable owner

Activity or decision	Board	SRO	Transformation Office	Workstream lead	Finance
Approve savings targets	**A**	**R**	C	C	C
Design workstream changes	I	**A**	C	**R**	C
Track savings weekly	I	I	**R**	C	**A**
Go or no-go after pilot	I	**A**	**R**	C	C
Report to Board	I	**A**	**R**	I	C

R Responsible **A** Accountable (one per row) **C** Consulted **I** Informed

Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Wave 1 benefits are on track except theatre cancellations

Benefit	Owner	Baseline	Target	Actual	Progress to target	Status	Due
Agency spend, pilot wards	Dir. of Nursing	210k/wk	120k/wk	168k/wk	**47%**	On track	Jun 27
Rosters published 6 weeks ahead	Dir. of Nursing	0%	95%	62%	**65%**	On track	Jun 27
On-the-day theatre cancellations	Surgical Dir.	14%	5%	12%	**22%**	Off track	Sep 27
Glove contracts	Head of Proc.	11	1	6	**50%**	At risk	Jan 28

Achieved · Remaining to target · On track · At risk · Off track

1. Actuals to October 2026 for the pilot wards only.

Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

The new surgical wing is at Gate 3; full business case approval is due in January



Strategic need

Mar 2025
Approved

Outline business case

Oct 2025
Approved, £430m

Design and procurement

Jun 2026
Approved with conditions

Full business case

Jan 2027

- Contractor price within £430m
- QRA at P80 funded
- Planning consent

Handover

Sep 2028

- Commissioning complete
- Benefits plan agreed

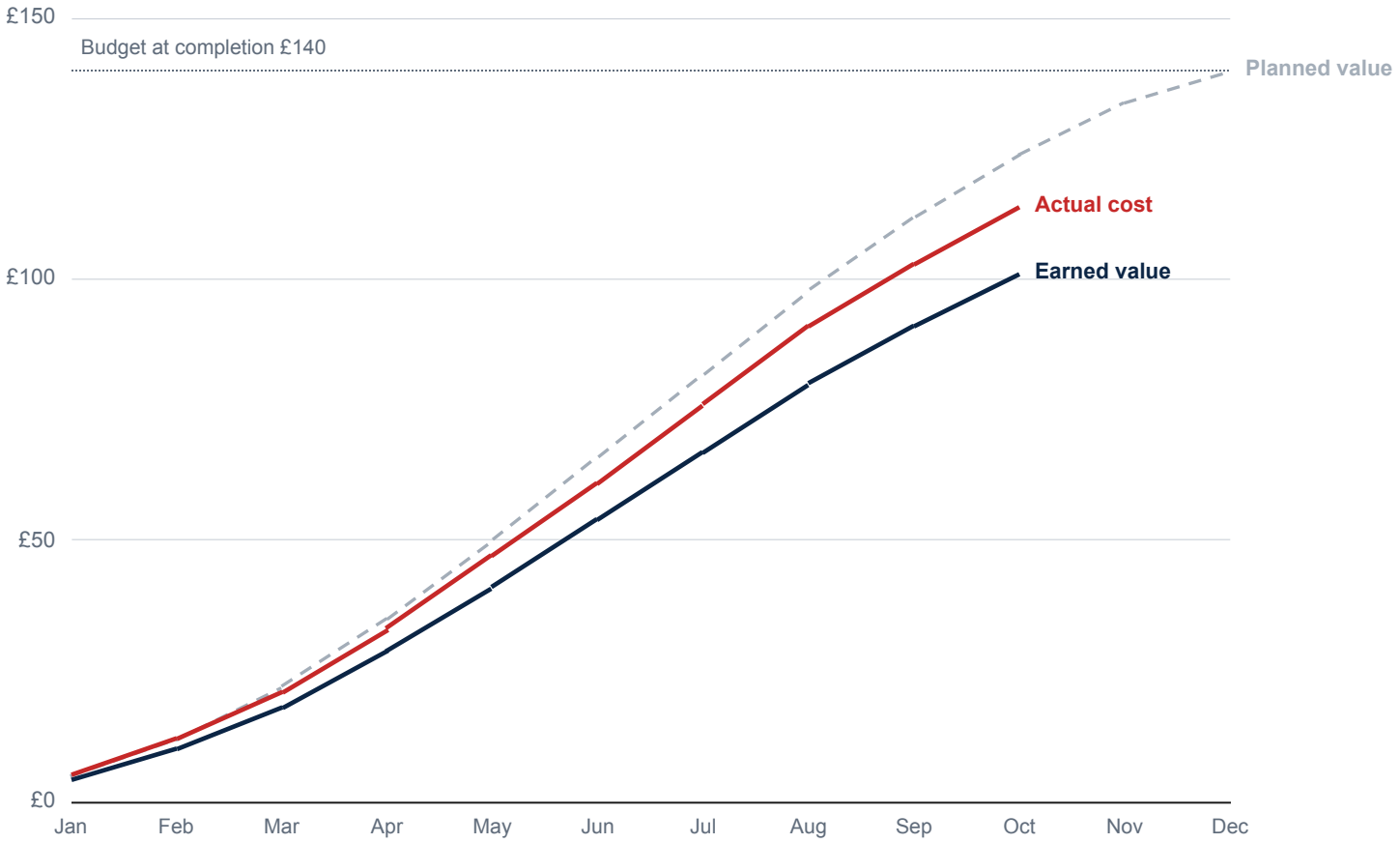
◆ Passed ◆ Current gate ◆ Future

1. Gates follow the group's capital investment policy.
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Construction has earned £101m of work against £124m planned, and each £1 spent has earned 89p

Earned value, cumulative

£m, nominal



Performance at Oct

SPI

0.81 ●

Schedule variance -£23

CPI

0.89 ●

Cost variance -£13

EAC

£158 ●

BAC / CPI; £18.0 over budget

Green 0.95 or more; amber 0.90-0.95; red below 0.90. Indices computed from the data shown.

What this shows

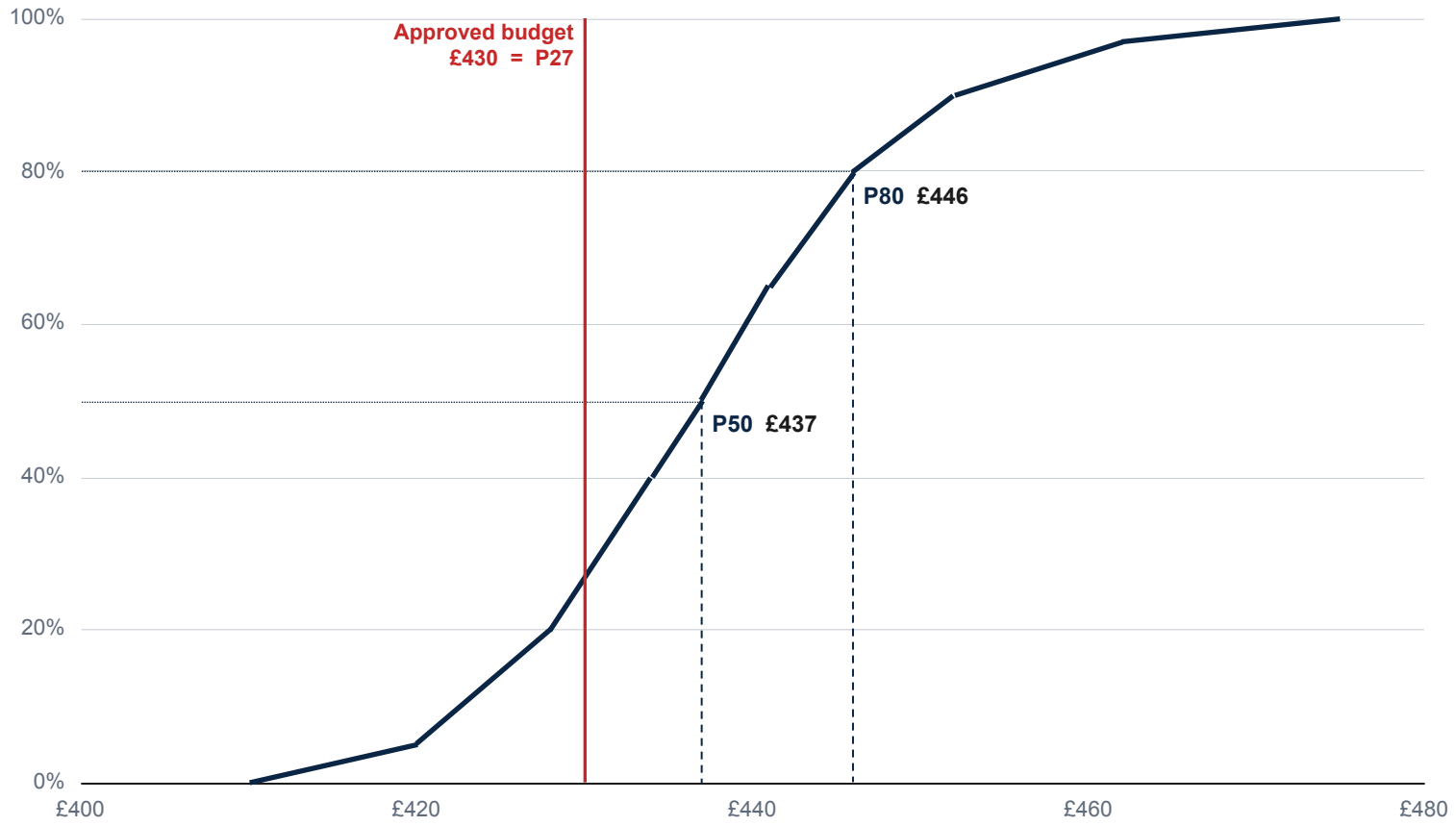
1. At this efficiency the phase finishes about **£18m** over its £140m budget.

1. Enabling works and main construction only. Cumulative from January 2026.
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

The £430m budget has only about a one-in-four chance of being enough

Probability that outturn cost is at or below

£m, nominal



Implication for Gate 3

1. Funding to P80 needs **£16m** more than approved.
2. The largest drivers are ground conditions and the M&E package.

1. Monte Carlo cost risk analysis, 5,000 iterations, October 2026 risk register.
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Open risks, issues and dependencies for the Board's attention

Type	ID	Description	Owner	Due	RAG	Status and next action
Risk	R04	Ground conditions worse than surveyed under Block B	Project director	Dec 26	🔴	Extra boreholes ordered; results 6 Dec
Risk	R09	M&E subcontractor capacity in 2027	Commercial lead	Jan 27	🟡	Second supplier being qualified
Issue	I02	Planning condition 14 (traffic) not yet discharged	Estates director	Nov 26	🟡	Submission made 2 Nov
Assumption	A01	Inflation of 4% a year on construction costs	Finance	Jan 27	🟡	Recheck against index at Gate 3
Dependency	D03	Regional funding for equipment (£22m)	CFO	Mar 27	🟢	Bid submitted; decision expected March

🟢 On track 🟡 At risk 🔴 Off track or overdue

Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Ground conditions and construction inflation drive most of the uncertainty in the wing's cost

Outturn cost of the surgical wing under low and high cases

£m, nominal



Implication

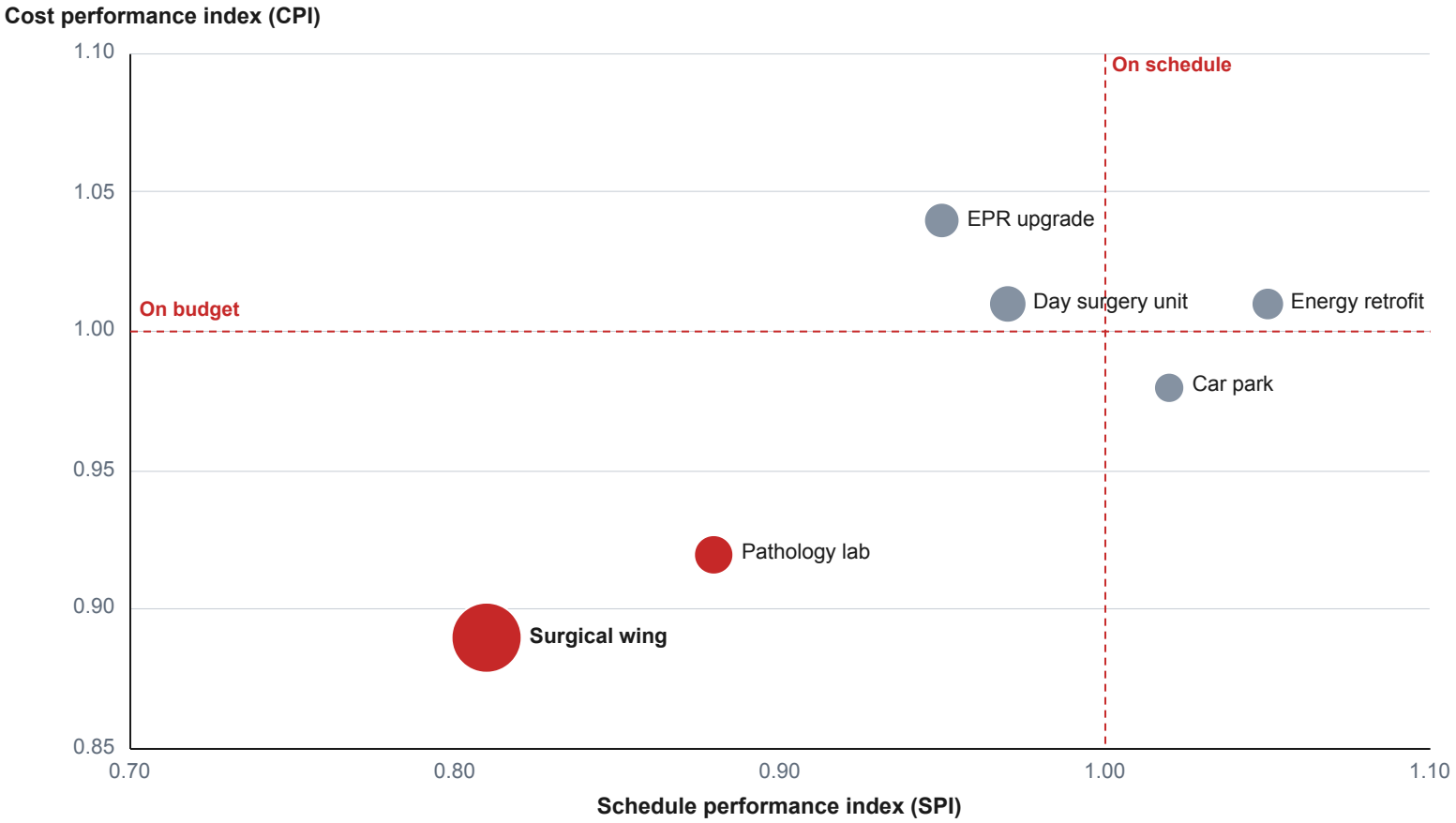
1. Resolving ground conditions before Gate 3 removes the largest single uncertainty.

1. Each bar varies one input across its range with all others at base case.
Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.

Two of the group's six capital projects are both late and over budget

Capital portfolio: schedule vs cost performance

October 2026



Implication

1. The surgical wing and pathology lab need recovery plans before the next Board.

1. Bubble size is approved budget.

Source: Meridian finance and activity data FY2025/26; PMO analysis. Fictional example.